REFERENCE SAFETY INFORMATION

Investigator's Brochure, Section 7 — kelvaparib
Kelvaparib (KLV-3308)
Marnholt Oncology GmbH

1. PURPOSE AND STATUS OF THIS SECTION

This section is the reference safety information for kelvaparib. It is the sole document against which the expectedness of an adverse reaction is determined for the purpose of expedited reporting. A reaction listed in the table in section 3 is expected; a reaction not listed there is unexpected, irrespective of whether it is described elsewhere in the Investigator's Brochure, in the company core data sheet, or in a periodic safety report.

Expectedness is judged on the term and on the severity stated. A reaction listed without a severity qualifier is expected at any severity. Where a maximum expected severity is stated, an event exceeding that severity is unexpected and is reportable on an expedited basis even though the term itself appears in the table.

This edition is effective from the date shown on each page. Reactions added or amended in this edition are identified in section 4. The date from which each change took effect is recorded in the trial master file, and expectedness for an individual case is determined against the edition in force on the date the sponsor became aware of the case.

2. BASIS OF THIS EDITION

This edition reflects the cumulative safety experience across the kelvaparib clinical development programme to the data lock point for the fifth annual development safety update report. It incorporates the addition of myelodysplastic syndrome and acute myeloid leukaemia as an expected adverse reaction, made on the basis of the cumulative incidence observed across the programme.

3. EXPECTED ADVERSE REACTIONS

The reactions below are expected for kelvaparib. Terms are given as MedDRA preferred terms within system organ class. Frequency categories describe the cumulative programme experience and are provided for information; they do not affect the determination of expectedness.

System organ class / Preferred term	Frequency	Maximum expected severity	Added in edition
Blood and lymphatic system disorders			
Anaemia	Very common	Grade 4	1
Thrombocytopenia	Very common	Grade 4	1
Neutropenia	Very common	Grade 4	1
Leukopenia	Common	Grade 3	1
Lymphopenia	Common	Grade 3	2
Neoplasms benign, malignant and unspecified			
Myelodysplastic syndrome	Common	Any	6
Acute myeloid leukaemia	Common	Any	6
Gastrointestinal disorders			
Nausea	Very common	Grade 3	1
Vomiting	Very common	Grade 3	1
Constipation	Very common	Grade 2	1
Abdominal pain	Very common	Grade 3	1
Diarrhoea	Very common	Grade 3	1
Dyspepsia	Very common	Grade 2	2
General disorders and administration site conditions			
Fatigue	Very common	Grade 3	1
Asthenia	Very common	Grade 3	1
Metabolism and nutrition disorders			
Decreased appetite	Very common	Grade 2	1
Respiratory, thoracic and mediastinal disorders			
Cough	Very common	Grade 2	1
Pneumonitis	Common	Grade 3	4
Psychiatric disorders			
Insomnia	Very common	Grade 2	2
Infections and infestations			
Urinary tract infection	Common	Grade 3	1

Frequency: very common at least 1 in 10; common at least 1 in 100 to less than 1 in 10; uncommon at least 1 in 1,000 to less than 1 in 100. Maximum expected severity is by CTCAE grade. "Any" means the reaction is expected at any severity including fatal.

4. CHANGES INTRODUCED IN THIS EDITION

Myelodysplastic syndrome and acute myeloid leukaemia have been added as expected adverse reactions at any severity, including fatal. The addition rests on the cumulative incidence across the programme, in which myelodysplastic syndrome or acute myeloid leukaemia was reported in 9 subjects (1.4 per cent) in study KLV-7701, together with the established association between inhibition of poly (ADP-ribose) polymerase and these events. Both are recorded as important identified risks in the periodic safety reporting for the programme.

The consequence of this addition is that a case of myelodysplastic syndrome or acute myeloid leukaemia arising on or after the effective date of this edition is expected and is therefore not reportable on an expedited basis on grounds of expectedness alone. A case arising before that date was unexpected and was reported on an expedited basis. This distinction should be applied by date of sponsor awareness and not retrospectively.

No other reaction has been added, removed or amended in this edition. The maximum expected severity for pneumonitis remains grade 3, so a grade 4 or fatal pneumonitis remains unexpected and reportable on an expedited basis.

5. REACTIONS UNDER EVALUATION AND NOT YET EXPECTED

The following are under evaluation and are not expected at the date of this edition. A case of any of them is unexpected and is reportable on an expedited basis. Second primary malignancy other than myelodysplastic syndrome or acute myeloid leukaemia. Effects on fertility. Both are recorded as important potential risks in the periodic safety reporting for the programme, and neither has an observed incidence sufficient to support addition to section 3.

6. RELATIONSHIP TO OTHER DOCUMENTS

The company core data sheet states the information the sponsor proposes for the product information at the time of a marketing application. It is not the reference safety information and must not be used to determine expectedness. Where the two documents differ, this section governs for expedited reporting and the core data sheet governs for the proposed product information. A reaction may be described in a warning in the core data sheet while remaining absent from section 3 above, in which case it is unexpected.